PHARMACOVIGILANCE INTERNSHIP

TASK 1: ADR IDENTIFICATION BASICS

Case-Based Assessment of a Published Patient Case Report

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- **Internship Name:** Pharmacovigilance Internship
- **Task Number:** Task-1
- **Task Title:** Adverse Drug Reaction (ADR) Identification Basics

1. Case Reference and Source

- **Title of published case report:** Amoxicillin-Clavulanate-Induced Pityriasis Rosea: A Pharmacovigilance Case Report
- **Author(s):** Cecil Franz [1], Divya Handa [1], Girish Joseph [1,✉], Neena Bhatti [1], Dinesh K Badyal [1]
- **Journal name:** Cureus (Part of Springer Nature)
- **Year of publication:** 2026
- **DOI / PMID:** https://doi.org/10.7759/cureus.102309
- **Source link:** https://pmc.ncbi.nlm.nih.gov/articles/PMC12934355/

2. Patient Demographic Information

Parameter	Details
Patient identifier (anonymized)	A 34 year old male with no name reported in case scenario of the published article
Age	34
Gender	Male
Relevant medical history	No relevant medical history
Previous drug allergies / ADRs	No previous drug allergies and ADR
Relevant pre-existing conditions	None

3. Medication History

3.1 Suspected Drug

Parameter	Details
Generic name	Amoxicillin Clauvanate
Brand name (if reported)	Not reported
Pharmacological class	**Beta Lactum Antibiotic**
Indication	After 48 hours from drug administration patient developed an erythematous rash localized at axillary region.
Dose and frequency	Fixed Dose Combination
Route of administration	Oral
Treatment start date	Not reported
Treatment stop date	Not reported

3.2 Concomitant Medication(s)

Medicine	Dose / Route	Indication	Start / Stop dates
None	-	-	-

4. Adverse Drug Reaction (ADR) Identification

Parameter	Details
Reported reaction / symptoms	Erythematous rashes from axillary region to flexural regions (groin and cubital fossae). Lesions were associated with intense pruritus
Clinical diagnosis	Cutaneous examination revealed multiple well-defined, oval erythematous plaques with mild peripheral scaling, involving a flexural area.
Suspected affected organ system	Skin (Integumentary system)
Date / time of onset	Not reported
Duration of reaction	2 days

Relevant laboratory / diagnostic findings	The lesions appeared symmetrical, slightly raised, and non-vesicular, with zero evidence of ulceration, crusting, or mucosal Involvement.
Treatment given for the reaction	topical corticosteroids and oral antihistamines
Reported outcome	Pityriasis rosea-like eruption occurred that were related to the drug Amoxicillin-Clauvanate.

5. Clinical Timeline

Event	Date / Time	Details
Before suspected drug administration	Not reported	No prior medical history of drug allergies and ADR
Initiation of suspected drug	Not reported	Patient was prescribed a fixed-dose combination of Amoxicillin-Clauvanate for an upper respiratory tract infection
First appearance of symptoms	Not reported	48 hours after initiating therapy, the patient developed an erythematous rash localized to the axillary region
Clinical evaluation / diagnosis	Not reported	Amoxicillin-Clauvanate was identified as the probable offending agent.
Drug withdrawal or dose modification	Not reported	The suspected drug was immediately discontinued
Treatment / intervention	Not reported	Topical corticosteroids and oral antihistamines
Follow-up and outcome	Not reported	In 10-14 days patient showed clinical improvement and on follow up there were no complications.

6. Assessment of the Drug–Reaction Relationship

6.1 Temporal Association:

The reaction occurred **48 hours** after the administration of Amoxicillin-Clauvanate in the patient, suggesting that **there was a Temporal relationship** of the drug with the reaction.

6.2 Dechallenge:

The suspected drug was **withdrawn immediately** after the suspected drug was identified. This **improved** the patient's lesions in 10-14 days and was completely treated by the next follow up. This **Established the De-challenge of the drug.**

6.3 Rechallenge:

Not reported in the article.

6.4 Alternative Causes:

Patients first consultation to the physician, the reaction was diagnosed as **scabies initially.** On treatment the **conditioned worsened**. So there were alternative causes considered but **no alternative causes** evidence were found.

6.5 Supporting Evidence

The Distribution and Morphology of the Lesions were indicating pityriasis rosea-like eruption, particularly in a drug-related context. There was also a Causality assessment done in the published article, using Naranjo Adverse Drug Reaction Probability Scale, which gave a score of 6, indicating that the reaction is a probable ADR

7. Preliminary ADR Classification

Assessment parameter	Finding
Suspected drug	Amoxicillin-Clauvanate
Identified reaction	Pityriasis Rosea

Drug-relatedness suspected?	Yes, Drug related reaction was suspected.
Reaction onset	48 hours
Dechallenge response	Improvement
Rechallenge evidence	Not reported
Alternative causes	No Alternative evidence in the report
Published causality result	Naranjo ADR probability scale: 6 (probable ADR)

8. Case Summary

- **Case summary:** A 34-year-old male developed an itchy and painful pityriasis rosea-like skin eruption approximately 48 hours after starting amoxicillin–clavulanate therapy. The suspected medicine was discontinued, and the patient received treatment with topical corticosteroids and oral antihistamines to manage the symptoms. The patient's skin condition subsequently improved and resolved. The case report assessed the association between amoxicillin–clavulanate and the skin reaction as probable, highlighting the importance of identifying and documenting suspected adverse drug reactions for patient safety.

9. Conclusion

- **Suspected medicine:** Amoxicillin-Clauvanate
- **Identified ADR:** Pityriasis Rosea
- **Key evidence supporting identification:** The Distribution and Morphology of the Lesions were indicating pityriasis rosea-like eruption, particularly in a drug-related context
- **Clinical outcome:** Pityriasis rosea-like eruption occurred that were related to the drug Amoxicillin-Clauvanate.
- **Important limitations or missing information:** The case was a single patient clinical observation that limited the generalization of the ADR with the drug. Dates, Patient name were missing of ADR reporting.

10. References

- Franz, C., Handa, D., Joseph, G., Bhatti, N., & Badyal, D. K. (2026). Amoxicillin-clavulanate-induced pityriasis rosea: A pharmacovigilance case report. Cureus, 18(1), e102309. https://doi.org/10.7759/cureus.102309.

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